Nursing Essentials

Priority: Keep glucose in the individualized target range while supporting nutrition, fetal surveillance, and safe birth planning.

FOCUS 1

Understand

- **Maternal effects:** increased risk of hypertensive disorders and later type 2 diabetes.
- **Fetal / newborn effects:** excessive growth, birth injury risk, and hypoglycemia after birth.
- **Screening:** routine testing is commonly at 24–28 weeks; earlier evaluation may assess previously unrecognized diabetes.

FOCUS 2

Manage

- **Track glucose:** fasting and postmeal checks as prescribed; document food, activity, and medication patterns.
- **Common targets:** fasting below 95 mg/dL; 1-hour postmeal below 140 or 2-hour below 120 mg/dL. Use the ordered goals.
- **Support:** individualized nutrition, appropriate activity, and medication when needed; insulin is commonly preferred when medication is required.

FOCUS 3

Protect

- **Hypoglycemia:** recognize sweating, shakiness, confusion, or other symptoms; check and treat using the prescribed plan.
- **Illness / poor intake:** contact the team about glucose, hydration, and medication adjustments; do not improvise dose changes.
- **After birth:** newborn glucose surveillance and early feeding follow the neonatal protocol.

Do not skip postpartum follow-up

Make it stick: After the cord is clamped, the maternal glucose supply stops, but the newborn's insulin effect can persist. That is why newborn hypoglycemia surveillance matters.

Focus	What to know	Nursing application
Recheck	a 75-g oral glucose tolerance test is generally recommended at 4–12 weeks postpartum.	Arrange the postpartum test
Continue screening	long-term diabetes surveillance remains important even if glucose normalizes.	Connect ongoing care
Teach	balanced meals, movement, future-pregnancy planning, and ongoing primary care.	Use teach-back

RED FLAGS · ESCALATE PROMPTLY

Severe hypoglycemia, altered mental status, persistent vomiting, dehydration, or concern for ketoacidosis requires urgent assessment.